



LABORATORY REPORT



Name : Mr. DHRISHIT SHAH	Sex/Age : Male / 16 Years	Case ID : 21000109741
Ref. By :	Dis. At :	Pt. ID : 215056
Bill. Loc. :		Pt. Loc. :
Reg Date and Time : 09-Oct-2022 08:52	Sample Type :	Mobile No. : 7819811953
Sample Date and Time : 09-Oct-2022 08:52	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time :	Acc. Remarks	Ref Id2 :

Abnormal Result(s) Summary

Test Name	Result Value	Unit	Reference Range
Lipid Profile			
HDL Cholesterol	29.00	mg/dL	40 - 60
Triglyceride	309.00	mg/dL	40 - 200
Test Remark: ? fasting sample. Adv : Repeat sample after proper 10 to 12 hour of fasting or after 1 week			
VLDL	61.80	mg/dL	10 - 40
Chol/HDL	6.59		0 - 4.1
LDL Cholesterol (Direct)	124.00	mg/dL	0 - 100
LDL Cholesterol	100.20	mg/dL	65 - 100

Abnormal Result(s) Summary End

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

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Name : Mr. DHRISHIT SHAH	Sex/Age : Male / 16 Years	Case ID : 21000109741
Ref. By :	Dis. At :	Pt. ID : 215056
Bill. Loc. :		Pt. Loc. :
Reg Date and Time : 09-Oct-2022 08:52	Sample Type : Serum	Mobile No. : 7819811953
Sample Date and Time : 09-Oct-2022 08:52	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 09-Oct-2022 11:28	Acc. Remarks	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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BIOCHEMICAL INVESTIGATIONS

Lipid Profile

Cholesterol	191.00	mg/dL	110 - 200	
HDL Cholesterol <i>Accelerator Selective Detergent</i>	L 29.00	mg/dL	40 - 60	
Triglyceride <i>Colorimetric-Arsenazo Method</i>	H 309.00	mg/dL	40 - 200	? fasting sample. Adv : Repeat sample after proper 10 to 12 hour of fasting or after 1 week
VLDL <i>Calculated</i>	H 61.80	mg/dL	10 - 40	
Chol/HDL <i>Calculated</i>	H 6.59		0 - 4.1	
LDL Cholesterol (Direct) <i>Enzymatic</i>	H 124.00	mg/dL	0 - 100	
LDL Cholesterol <i>Calculated</i>	H 100.20	mg/dL	65 - 100	

NEW ATP III GUIDELINES (MAY 2001), MODIFICATION OF NCEP

LDL CHOLESTEROL	CHOLESTEROL	HDL CHOLESTEROL	TRIGLYCERIDES
Optimal <100	Desirable <200	Low <40	Normal <150
Near Optimal 100-129	Border Line 200-239	High >60	Border High 150-199
Borderline 130-159	High >240	-	High 200-499
High 160-189	-	-	-

- LDL Cholesterol level is primary goal for treatment and varies with risk category and assesment
- For LDL Cholesterol level Please consider direct LDL value
- Risk assesment from HDL and Triglyceride has been revised. Also LDL goals have changed.
- Detail test interpretation available from the lab
- All tests are done according to NCEP guidelines and with FDA approved kits.
- LDL Cholesterol level is primary goal for treatment and varies with risk category and assesment

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

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Name : Mr. DHRISHIT SHAH	Sex/Age : Male / 16 Years	Case ID : 21000109741
Ref. By :	Dis. At :	Pt. ID : 215056
Bill. Loc. :		Pt. Loc. :
Reg Date and Time : 09-Oct-2022 08:52	Sample Type : Whole Blood EDTA	Mobile No. : 7819811953
Sample Date and Time : 09-Oct-2022 08:52	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 09-Oct-2022 10:43	Acc. Remarks	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
BIOCHEMICAL INVESTIGATIONS				
Glycated Haemoglobin Estimation				

HbA1C HPLC	5.30	% of total Hb	<5.7: Normal 5.7-6.4: Prediabetes >=6.5: Diabetes
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Avg. Pl Glucose (Last 3 Months) Calculated	105.41	mg/dL	80.00 - 140.00
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Please Note change in reference range as per ADA 2021 guidelines.

Interpretation :

HbA1C level reflects the mean glucose concentration over previous 8-12 weeks and provides better indication of long term glycemic control.

Levels of HbA1C may be low as result of shortened RBC life span in case of hemolytic anemia.

Increased HbA1C values may be found in patients with polycythemia or post splenectomy patients.

Patients with Homozygous forms of rare variant Hb(CC,SS,EE,SC) HbA1c can not be quantitated as there is no HbA.

In such circumstances glycemic control can be monitored using plasma glucose levels or serum Fructosamine.

The A1c target should be individualized based on numerous factors, such as age, life expectancy, comorbid conditions, duration of diabetes, risk of hypoglycemia or adverse consequences from hypoglycemia, patient motivation and adherence.

----- End Of Report -----

For test performed on specimens received or collected from non-NSRL locations, it is presumed that the specimen belongs to the patient named or identified as labeled on the container/test request and such verification has been carried out at the point generation of the said specimen by the sender. NSRL will be responsible Only for the analytical part of test carried out. All other responsibility will be of referring Laboratory.

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

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